ASCP / CAP-ACP 2026 · MONTRÉAL · POSTER 6 · HEMATOPATHOLOGY

Oligoclonal CAR-T–Derived CD4+ T-Cell Lymphoproliferative Disorder of the Duodenum Following BCMA-Directed CAR-T Therapy for Multiple Myeloma

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COMPANION SITE
original electropherograms, full references

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BACKGROUND

Immune effector cell–associated enterocolitis (IEC-EC) was first described in patients receiving BCMA-directed CAR-T therapy for multiple myeloma, who developed severe non-infectious diarrhea months after infusion (Fortuna 2024). Biopsies showed a duodenum-predominant pattern of injury — villous atrophy, intraepithelial lymphocytosis and loss of plasma cells — that responds to immunosuppression (Osnis 2026; Karimi 2025; Banerjee 2025; Blumenberg 2026). CAR-positive T cells, many highly cytotoxic, persist in the mucosa (Ritter 2026; Kethidi 2026), but these lesions were regarded as inflammatory.

Ozdemirli et al. first proposed that the infused product itself could become malignant, reporting an indolent CD4+ T-cell lymphoma of the small intestine that carried the CAR transgene (2024). It followed the FDA's class-wide warning on T-cell malignancies after CAR-T (Verdun 2024), and further transgene-positive lymphomas were reported, several arising from pre-existing mutated clones or carrying the vector in a tumor suppressor (Perica, Harrison, Braun, Aleman 2025; Shi 2026). Proposed names include *CAR transgenic T-cell lymphoproliferative neoplasm* (Baumgartner 2026) and, for indolent mucosal disease, *indolent intestinal CAR T-cell lymphoproliferative disorder* (manuscript in revision).

Post-CAR-T gut disease is now read as a spectrum: non-CAR-mediated injury, polyclonal CAR T-cell enteritis, and clonal CAR T-cell LPD (Somay 2025; Ali 2026; Sarami 2026). Clonal cases carry the most severe enteritis and the highest mortality (Patel 2026), yet construct-positive mucosa alone is not LPD (Susanibar-Adaniya 2026), and a clonal, vector-integrated expansion that was not malignant has been described (Guiraud 2026).

> Construct-specific (camelid V$_H$H) immunohistochemistry and TCR-γ clonality are available at most centers. **A framework for reading them together is not.**

CLINICAL COURSE

A 74-year-old man with IgG-λ multiple myeloma with *CCND1::IGH*, presenting four years before CAR-T with a pathologic C1 fracture, received Dara-RVd, a melphalan-conditioned autologous transplant and lenalidomide maintenance, then DKd at relapse. He received ciltacabtagene autoleucel (day 0) after fludarabine–cyclophosphamide lymphodepletion, without cytokine release syndrome or neurotoxicity, and reached an MRD-negative complete remission by clonoSEQ and PET/CT. About three months later he developed progressive diarrhea (more than ten stools daily) and a 20-lb weight loss; infectious studies were negative and the mucosa was endoscopically normal. Duodenal biopsies on days +141 and +190 showed a lamina propria T-cell infiltrate (Panels 01–03). Budesonide, cholestyramine, immunoglobulin and vedolizumab gave partial, transient benefit; ruxolitinib from day +228 restored oral intake. Norovirus with pneumatosis on day +245 led to a negative exploratory laparotomy. A third biopsy on day +303, **after symptomatic improvement**, showed persistent involvement of the duodenum and duodenal bulb. Cyclosporine from day +315, escalated to 450 mg twice daily, resolved the diarrhea; at day +413 he was gaining weight, with normal protein studies and no evidence of myeloma.

TIMELINE · DAYS FROM INFUSION

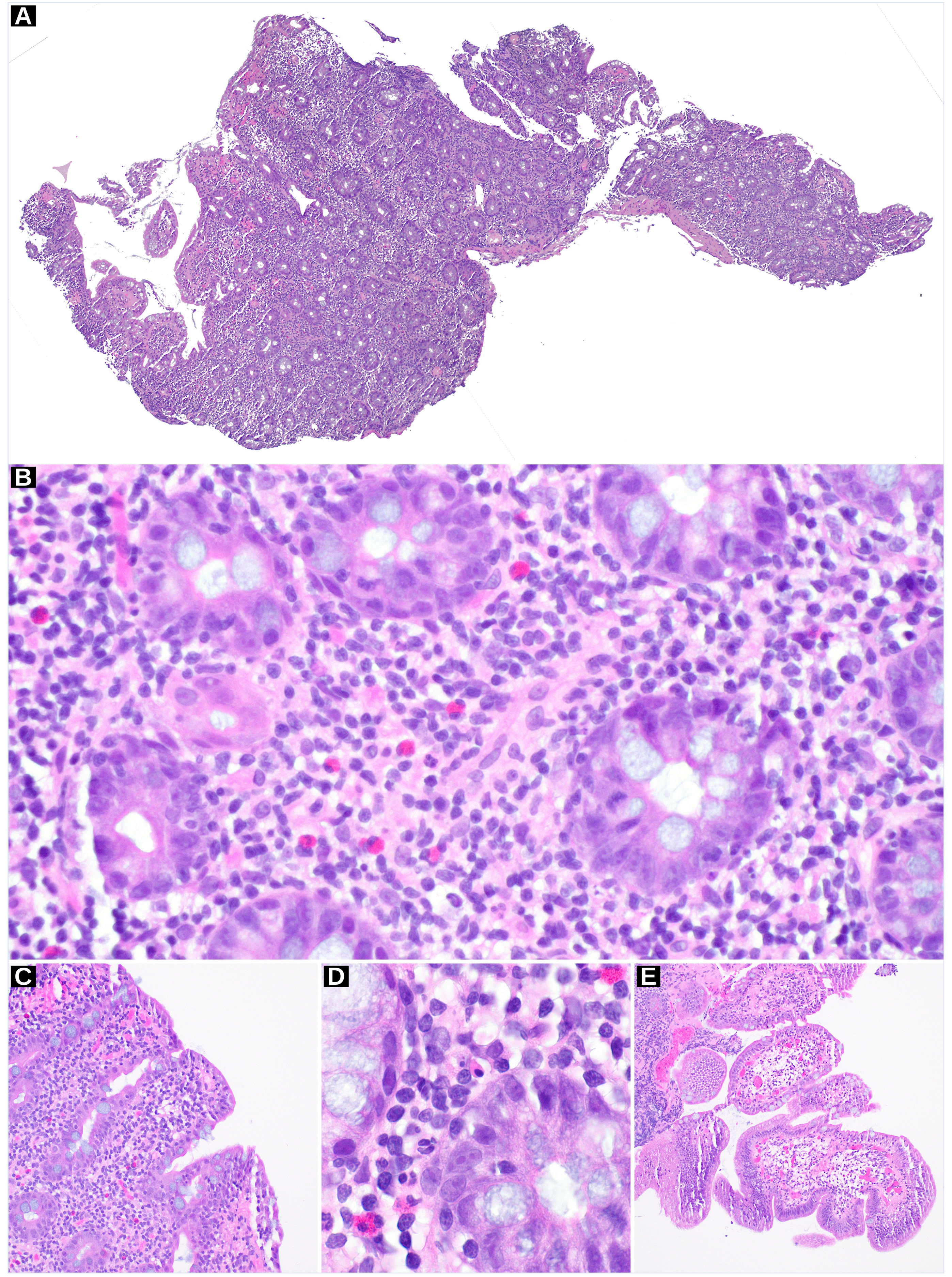
DAY 0	Ciltacabtagene autoleucel infused; no cytokine release syndrome or neurotoxicity.
+99	Marrow MRD-negative by clonoSEQ; PET/CT without avid disease.
+141	**First duodenal biopsy** for progressive diarrhea and weight loss; infectious workup negative.
+190	**Second biopsy.** Mucosa endoscopically normal on both occasions.
+193 → +224	Immunoglobulin, then vedolizumab; budesonide and cholestyramine earlier. Partial, transient responses.
+228	Ruxolitinib: symptoms improved, oral intake restored.
+245	Norovirus with pneumatosis; laparotomy for suspected perforation found none.
+303	**Third biopsy,** after symptomatic improvement: duodenum *and* duodenal bulb involved.
+315	Cyclosporine started, later escalated to 450 mg twice daily.
+413	Diarrhea resolved, weight climbing; protein studies normal, myeloma in remission.

ANCILLARY STUDIES

Anti-camelid V$_H$H	CAR construct in ~70% of infiltrating cells; **robust lamina propria positivity at all three biopsies**
TCR-γ PCR	**Oligoclonal**, the same bands at +141, +190 and +303
Targeted NGS	No relevant variants
TRBC1 flow, blood	No circulating clone
EBER in situ	Negative

01 HISTOPATHOLOGY

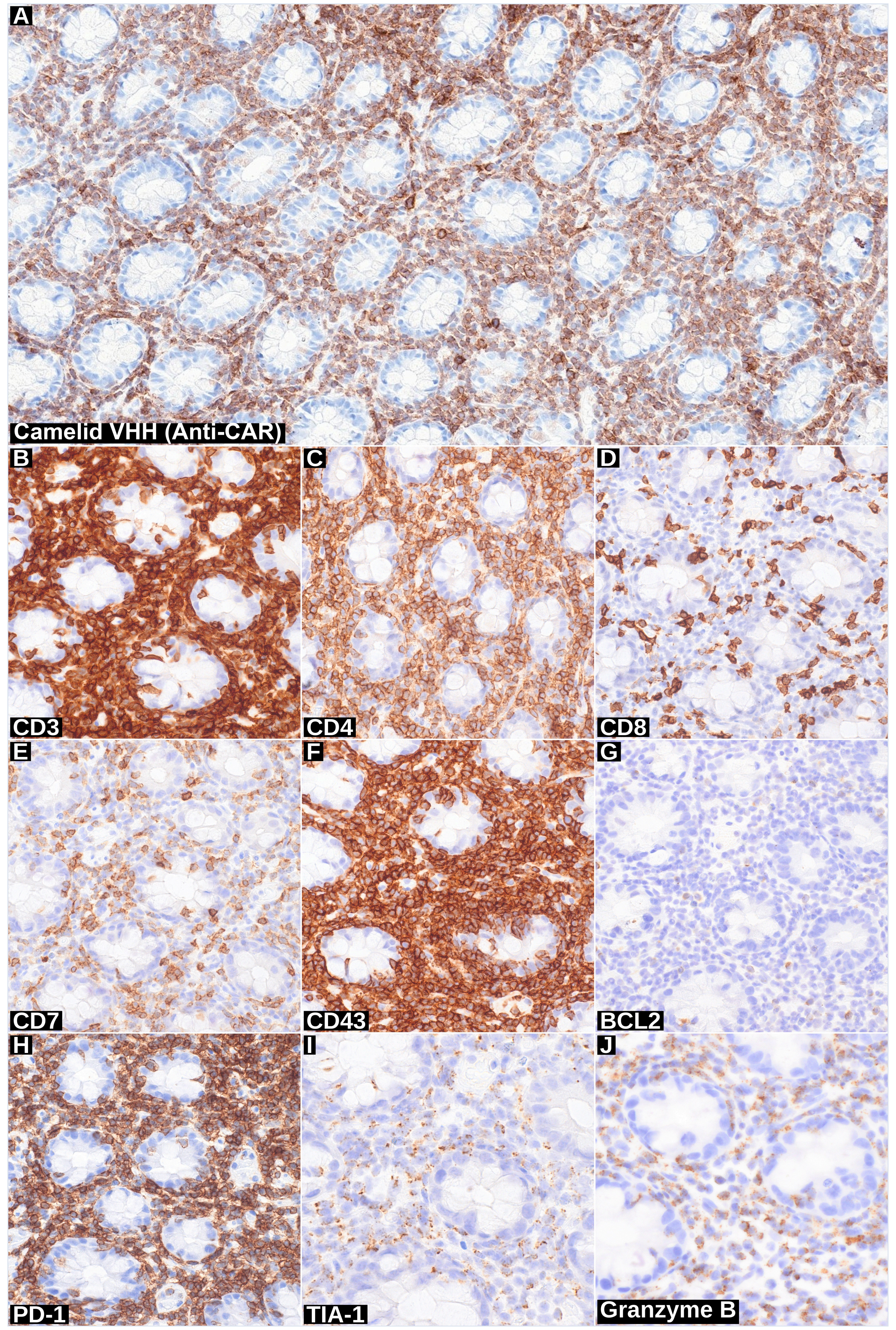
Duodenal biopsies, hematoxylin and eosin



A, Low-power overview: a dense lymphocytic infiltrate expands the lamina propria with relative preservation of glandular architecture. **B**, Dense, monotonous lymphoid infiltrate with increased intraepithelial lymphocytes. **C**, Villous blunting with a prominent lamina propria infiltrate. **D**, Apoptotic bodies within glandular epithelium. **E**, Foveolar metaplasia of the surface epithelium.

02 IMMUNOPHENOTYPE

Immunohistochemistry, including CAR-specific staining



A, Camelid V$_H$H (anti-CAR) immunohistochemistry confirms construct expression in the majority of infiltrating T cells. **B–D**, CD3 highlights the dense infiltrate; CD4 is positive in most cells, CD8 in a minor subset. **E–G**, CD7 is lost, CD43 diffusely positive, BCL2 negative. **H–J**, PD-1 positive; TIA-1 and granzyme B mark cytotoxic differentiation.

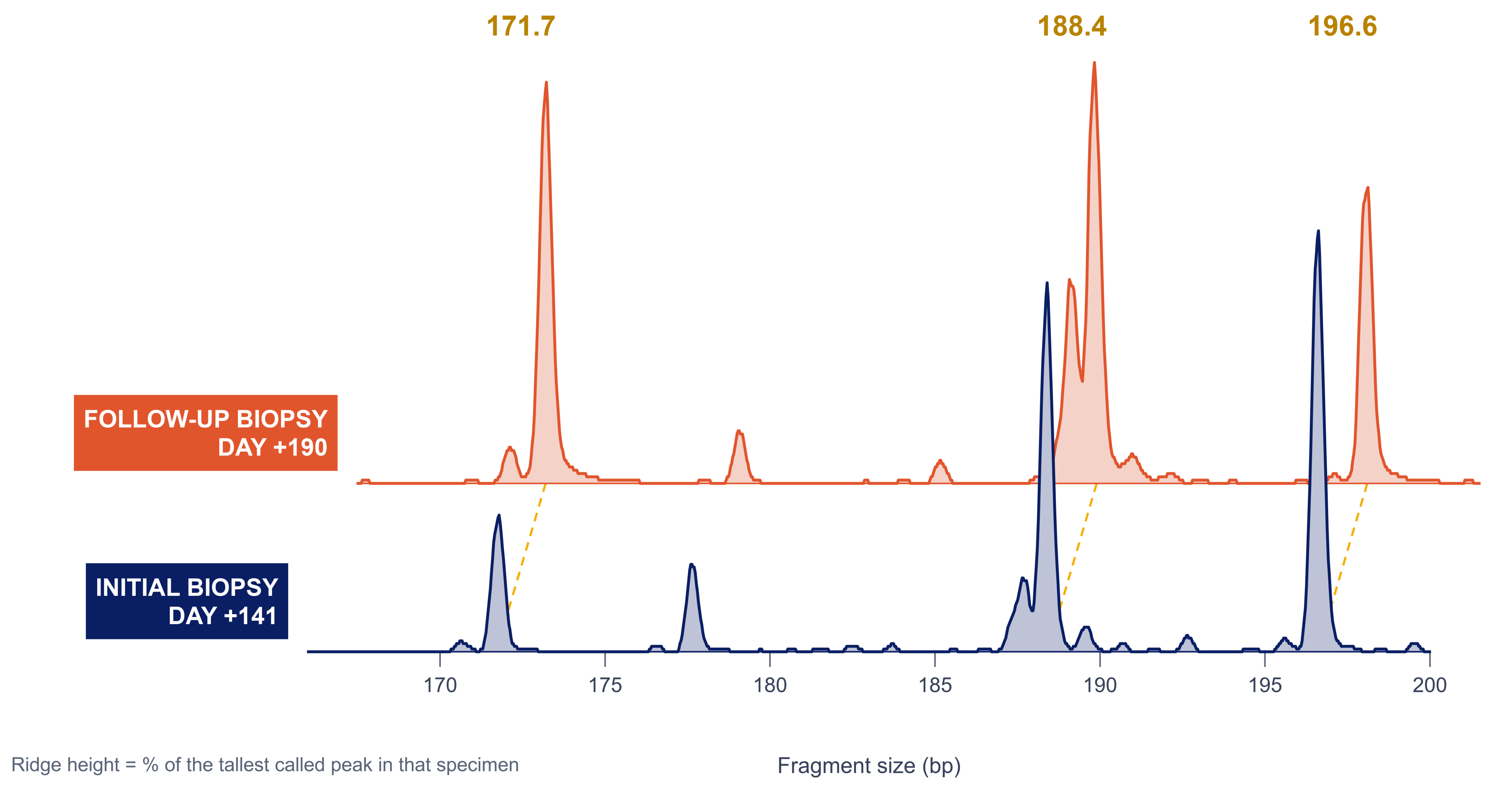
POSITIVE
CD3 · CD4 · CD5 · CD43 · PD-1 · TIA-1 · granzyme B · camelid V$_H$H (anti-CAR) · Ki-67 <10%

LOST OR NEGATIVE
CD7 (loss) · CD8 (minor subset only) · BCL2 · EBER ISH

> Across all three biopsies — days +141, +190 and +303 — the histology, the immunophenotype and the CAR positivity were the same: a dense lamina propria T-cell infiltrate with CD7 loss and robust camelid V$_H$H staining. **The lesion looked no different in the specimen taken after the symptoms had improved.**

03 TCR-γ CLONALITY ON SERIAL BIOPSIES

TCR-γ multiplex PCR with capillary electrophoresis · re-drawn from the laboratory trace · follow-up biopsy behind, initial biopsy in front on a shared size axis



Gold guides join the three peaks shared by both specimens (171.7, 188.4, 196.6 bp), recurring at the same sizes fifty days apart; 171.7 bp rises to co-dominance at day +190 and a minor 183.6 bp peak appears. **The third biopsy at day +303 reproduced the same bands again. Original instrument electropherograms for all four primer sets are on the companion site.**

05 FEATURES OF IEC-EC AND CLONAL CAR T-CELL LPD

Histologic features reported for IEC-EC and for CAR-derived gastrointestinal lymphoproliferation, from the primary literature · tissue series remain small; the clonal pole rests on case reports and small series

FAVORS IEC-EC

inflammatory pole · 27 patients with tissue[1,2,5]

- Apoptosis-dominant, GVHD-like injury — **7 bodies/10 hpf** duodenum, **9.5/10 hpf** ileum[1]
- Crypt dropout and neuroendocrine cell nests in colon[2]
- Foveolar metaplasia[1]
- Infiltrate scattered and mucosa-limited; no expansile, destructive mass
- **CAR-positive cells few or scattered**[5,7]
- Non-clonal; partial pattern, milder, transient course[8]

PRESENT IN BOTH

the overlap is nearly the whole picture

- Duodenum-predominant injury; colon relatively spared[1]
- Villous atrophy and blunting[1,2,5]
- Increased intraepithelial lymphocytes[1,2,5]
- Lamina propria T-cell infiltrate[1,6]
- Plasma cells reduced or absent — **25 of 26 specimens**[1,2]
- Delayed onset, median **92.5 days**; infection excluded[5]
- Responds to steroids, biologics, ruxolitinib or cyclosporine[3,4,9]

FAVORS CAR-T LPD

lymphoproliferative pole · case reports and small series

- Dense, expansile lamina propria infiltrate
- **CAR-positive cells ≥30% of T cells** — ~70% here
- T-cell antigen loss (CD7) despite bland cytology
- Aberrant CD4+ cytotoxic phenotype — TIA-1+, granzyme B+[6]
- **Clonal TCR-γ recurring at identical fragment sizes on serial biopsy**
- Lesion unchanged after the symptoms resolve — this case
- Ki-67 low: a persistent clone, not a proliferative one

Sources: [1]Osnis 2026 *Am J Surg Pathol* (10 pts, 26 bx) · [2]Karimi 2025 *Hum Pathol* (3 pts, 5 bx) · [3]Blumenberg 2026 *Blood* (5 pts) · [4]Banerjee 2025 *Blood Cancer J* · [5]Fortuna 2024 *Blood Cancer J* (14 pts) · [6]Ozdemirli 2024 *NEJM* · [7]Ritter 2026 *Leuk Lymphoma* · [8]Patel 2026 *Hum Pathol* (11 pts, 5 clonal) · [9]Hosoya 2026 *Blood*

04 CONCLUSIONS

- No single biopsy settled this lesion into enterocolitis or lymphoproliferative disorder. It took three specimens over ten months to declare itself.
- **Symptomatic improvement did not clear it.** The same oligoclonal bands, the same aberrant phenotype (CD7 loss; PD-1, TIA-1 and granzyme B on CD4+ T cells) and robust CAR positivity were present at every biopsy, including the one taken after the diarrhea settled.
- It never disseminated, the myeloma stayed in remission, and cyclosporine holds it. The lesion sat between the poles and stayed there.
- **TCR-γ clonality and construct-specific (camelid V$_H$H) immunohistochemistry belong on every post-CAR-T gastrointestinal biopsy in a myeloma patient** — the standard panel, not a reflex for atypical cases: 2 of 9 IEC-EC workups showed a T-cell LPD pattern (Lim 2026).
- The course fits **indolent intestinal CAR T-cell lymphoproliferative disorder**: localized, construct-positive, clonally stable over serial biopsies, suppressed rather than eradicated by immunosuppression.

References (full citations on the companion site)**:** Ozdemirli 2024 *NEJM* · Verdun 2024 *NEJM* · Perica 2025 *NEJM* · Harrison 2025 *NEJM* · Braun 2025 *Nat Med* · Aleman 2025 *NEJM* · Somay 2025 *Blood Cancer J* · Ali 2026 *Nat Rev Clin Oncol* · Baumgartner 2026 *Blood* · Shi 2026 *Blood Adv* · Osnis 2026 *Am J Surg Pathol* · Sarami 2026 *Hum Pathol* · Lim 2026 *Blood Adv* · Ritter 2026 *Leuk Lymphoma* · Guiraud 2026 *Haematologica* · Patel 2026 *Hum Pathol* · Kethidi 2026 *Nat Med* · Susanibar-Adaniya 2026 *Blood Cancer J* · Hosoya 2026 *Blood*

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